

Name : Rachel Ujallah
Roll no: 2196
Class : BS Psychology

- **Sleep wake disorder:**

- **Insomnia Disorders:**

Primary Insomnia: Difficulty initiating or maintaining sleep, causing distress or impairment in social, occupational, or other important areas of functioning.

Secondary Insomnia: Caused by another medical condition, substance use, or mental health disorder.

Chronic Insomnia: Lasts for at least three months, occurring at least three nights per week.

- **Hypersomnolence Disorders:**

Narcolepsy: Excessive daytime sleepiness, sudden loss of muscle tone (cataplexy), hallucinations, and sleep paralysis.

Idiopathic Hypersomnia: Excessive daytime sleepiness despite adequate nocturnal sleep, without cataplexy or other features of narcolepsy.

Circadian Rhythm Sleep-Wake Disorders:

Delayed Sleep Phase Disorder: Persistent difficulty falling asleep and waking up at socially acceptable times.

Advanced Sleep Phase Disorder: Falling asleep and waking up earlier than desired.

Irregular Sleep-Wake Rhythm Disorder: Lack of a regular sleep-wake pattern.

Shift Work Sleep Disorder: Insomnia or excessive sleepiness due to working non-traditional hours.

- **Parasomnia Disorders:**

Sleepwalking (Somnambulism): Walking or performing other complex behaviors while asleep.

Night Terrors: Abrupt awakening from sleep with intense fear, often accompanied by screaming or crying.

REM Sleep Behavior Disorder: Acting out vivid dreams during REM sleep, potentially causing injury to oneself or others.

Obstructive Sleep Apnea: Partial or complete obstruction of the upper airway during sleep, leading to pauses in breathing and disrupted sleep.

Central Sleep Apnea: Lack of respiratory effort during sleep, resulting in pauses in breathing.

Sleep-Related Hypoventilation Disorders: Abnormally low ventilation during sleep, leading to elevated carbon dioxide levels in the blood.

- **Treatment of Sleep-Wake Disorders:**

Medications:

Insomnia: Sedative-hypnotic medications like benzodiazepines, non-benzodiazepine hypnotics (e.g., zolpidem), or melatonin receptor agonists.

Narcolepsy: Stimulants (e.g., modafinil, methylphenidate) to promote wakefulness, sodium oxybate to reduce cataplexy and improve nighttime sleep.

Sleep Apnea: Continuous positive airway pressure (CPAP) therapy, oral appliances, or surgery in severe cases.

Behavioral Therapies:

Cognitive Behavioral Therapy for Insomnia (CBT-I): Addresses thoughts and behaviors that contribute to sleep difficulties.

Sleep Hygiene Education: Promotes healthy sleep habits, such as maintaining a consistent sleep schedule and creating a conducive sleep environment.

Relaxation Techniques: Meditation, progressive muscle relaxation, or deep breathing exercises to promote relaxation before bedtime.

Lifestyle Changes:

Diet and Exercise: Regular physical activity and a balanced diet can improve sleep quality.

Stress Management: Techniques such as mindfulness or stress-reduction exercises can help alleviate sleep disturbances caused by stress.

- **Assessment Tools for Sleep-Wake Disorders:**

Sleep Diaries:

Patients record their sleep patterns, including bedtime, wake time, sleep duration, and quality, over a specified period.

Actigraphy:

Wearable devices (actigraphs) measure movement to estimate sleep-wake patterns over several days or weeks.

Polysomnography (PSG):

Comprehensive sleep study conducted in a sleep laboratory, measuring brain waves, eye movements, muscle activity, heart rate, and breathing during sleep.

Questionnaires:

Epworth Sleepiness Scale: Assesses daytime sleepiness by rating the likelihood of falling asleep in different situations.

Berlin Questionnaire: Screens for risk factors associated with sleep apnea, such as snoring and daytime fatigue.

STOP-BANG Questionnaire: Identifies individuals at high risk for obstructive sleep apnea based on snoring, tiredness, observed apneas, blood pressure, BMI, age, neck circumference, and gender.

- **Causes of Sleep-Wake Disorders:**

Genetic Factors:

Some sleep disorders, such as narcolepsy and certain circadian rhythm disorders, have a genetic predisposition.

Medical Conditions:

Chronic conditions like asthma, arthritis, or gastrointestinal disorders can disrupt sleep. Neurological disorders such as Parkinson's disease or Alzheimer's disease may affect sleep-wake patterns.

Mental Health Disorders:

Anxiety, depression, bipolar disorder, and post-traumatic stress disorder (PTSD) can all impact sleep quality and duration.

Substance Use:

Alcohol, caffeine, nicotine, and certain medications can interfere with sleep.

Environmental Factors:

Shift work, irregular sleep schedules, noise, light exposure, and uncomfortable sleeping conditions can disrupt sleep.

Understanding these factors is crucial for accurate diagnosis and effective management of sleep-wake disorders. Treatment often involves a multidisciplinary approach, addressing both underlying causes and symptoms of the disorder.

